MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 2 OF 28

MFT 702

Theories of Family Therapy

14 teaching weeks · 6 practicals · 5 worked cases · 6 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701

> “Nine major schools, each treated as an answer to a specific question.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 702 — Theories of Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	**Theories of Family Therapy**	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Nine major schools, each treated as an answer to a specific question. You will learn each model well enough to do a first session in it, critique it, and integrate it — and you will practise moving between models on the same family until the choice becomes deliberate rather than habitual.

How to use this book

Theory is learned in three passes. **Pass one:** understand the model's core question and its intervention. **Pass two:** apply it to a case you know. **Pass three:** critique it and ask what it cannot see. Each week in this book gives you all three, plus the practical that makes the pass real. Do not memorise lists of techniques; memorise what problem each model believes it is solving.

1. Course description

There is no single "correct" family therapy. There are traditions, each with a theory of how problems develop, a theory of change, a preferred focus of attention, and a characteristic set of techniques. A clinician who knows only one model does not have a theory; they have a habit.

This course therefore pursues three aims: **comparative competence** (you can describe and demonstrate each major model), **critical appraisal** (you can name each model's evidence base and its blind spots), and **integration** (you can combine two models coherently in one case rather than dithering between them).

Organising question. For each model, ask: What does this model believe is the problem, and what does it therefore do first?

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the theoretical foundations, core concepts and change mechanisms of nine major models	Exam
LO2	Demonstrate a model-consistent first session for at least three models	Skills assessment
LO3	Compare models on focus, technique, role of the therapist and evidence	Comparative matrix
LO4	Identify the blind spots and cultural assumptions of each model	Essay
LO5	Construct a coherent integrative plan for a single case using two models	Case assignment
LO6	Justify model choice with reference to the client, the evidence and your own competence	Viva

3. Assessment

Component	Weight	Due
Comparative matrix (nine models × six dimensions)	20%	Week 7
Observed model-consistent session (20 min)	20%	Weeks 8–10
Integrative case study (2,500 words)	30%	Week 12
Written examination	30%	Week 15

4. The fourteen-week plan

Week	Model / topic	Practical	Key text
1	How to compare theories: six dimensions	Matrix construction	Nichols ch. 1
2	Psychoanalytic and object-relations family therapy	Genogram + interpretation exercise	Scharff & Scharff
3	Bowen's multigenerational theory	Detriangling role-play	Bowen (1978)
4	Structural family therapy	Enactment and boundary-making	Minuchin (1974)
5	Strategic and MRI brief therapy	Directive design	Haley & Madanes
6	Milan systemic therapy	Circular questioning	Palazzoli et al. (1980)
7	Experiential and human validation (Satir, Whitaker)	Sculpture exercise	Satir (1988)
8	Cognitive-behavioural and behavioural couple therapy	Behavioural exchange design	Epstein & Baucom
9	Emotionally focused therapy	EFT enactment	Johnson (2019)
10	Solution-focused brief therapy	Miracle question and scaling	de Shazer (1985)
11	Narrative therapy	Externalising conversation	White (2007)
12	Integration: combine two models without incoherence	Case integration workshop	Lebow (2014)
13	Evidence, culture and critique of each model	Debate	Sprenkle et al.
14	Your theoretical position	Position paper presentation	—

5. The six dimensions of comparison

Every model can be described on the same six axes. Learn to fill this grid from memory; it is the spine of the exam.

Dimension	The question it answers
Theory of problem	How does this model believe problems develop and persist?

Dimension	The question it answers
Unit of focus	Individual, dyad, family structure, meaning system, or context?
Theory of change	What must happen for things to improve?
Therapist position	Director, coach, collaborator, witness, expert?
Signature techniques	What does this model characteristically do?
Evidence base	What is the strength and quality of supporting research?

> **PRACTICAL**
> Build the matrix in a spreadsheet on day one. Add a row each week. By Week 7 you will have a revision document worth more than any summary anyone could give you.

Week 2 — Psychoanalytic and object-relations family therapy

The question it asks. What unconscious processes, carried from earlier relationships, are being re-enacted in this family now?

Core concepts. Object relations (internal working models of self and other formed in early relationships); projection and projective identification (disowned parts of the self are located in another person); splitting (people experienced as all-good or all-bad); repetition compulsion; the family as a shared unconscious system.

Therapy. Longer-term; the therapist attends to transference and countertransference, interprets patterns, and helps the family "hold" difficult feelings rather than evacuate them into one member. Frameworks include group-analytic family therapy and the Tavistock tradition.

Signature technique — interpretation, used sparingly. "I notice that whenever your mother speaks about her own mother, you begin to speak for her. I wonder whether that is how the family has always managed her sadness."

Contribution. It gave the field its respect for depth, for what is not said, and for the therapist's own emotional response as data.

Blind spots. Long, expensive, thin evidence base in its classical form; risks pathologising parents; the language can feel blaming ("the intrusive mother") and can ignore material context.

> **CASE**
> A young man cannot keep a job; his mother calls his employers before he does. Object-relations reading: the mother's own abandonment anxiety is projected into her son's incompetence, and his failure keeps her needed. Interpretation may be less useful than an experiment: agree that she will not call, and predict that both will feel anxious for two weeks. The prediction turns insight into something testable.

Week 3 — Bowen's multigenerational theory

The question. How is anxiety transmitted across generations, and who in the system absorbs it?

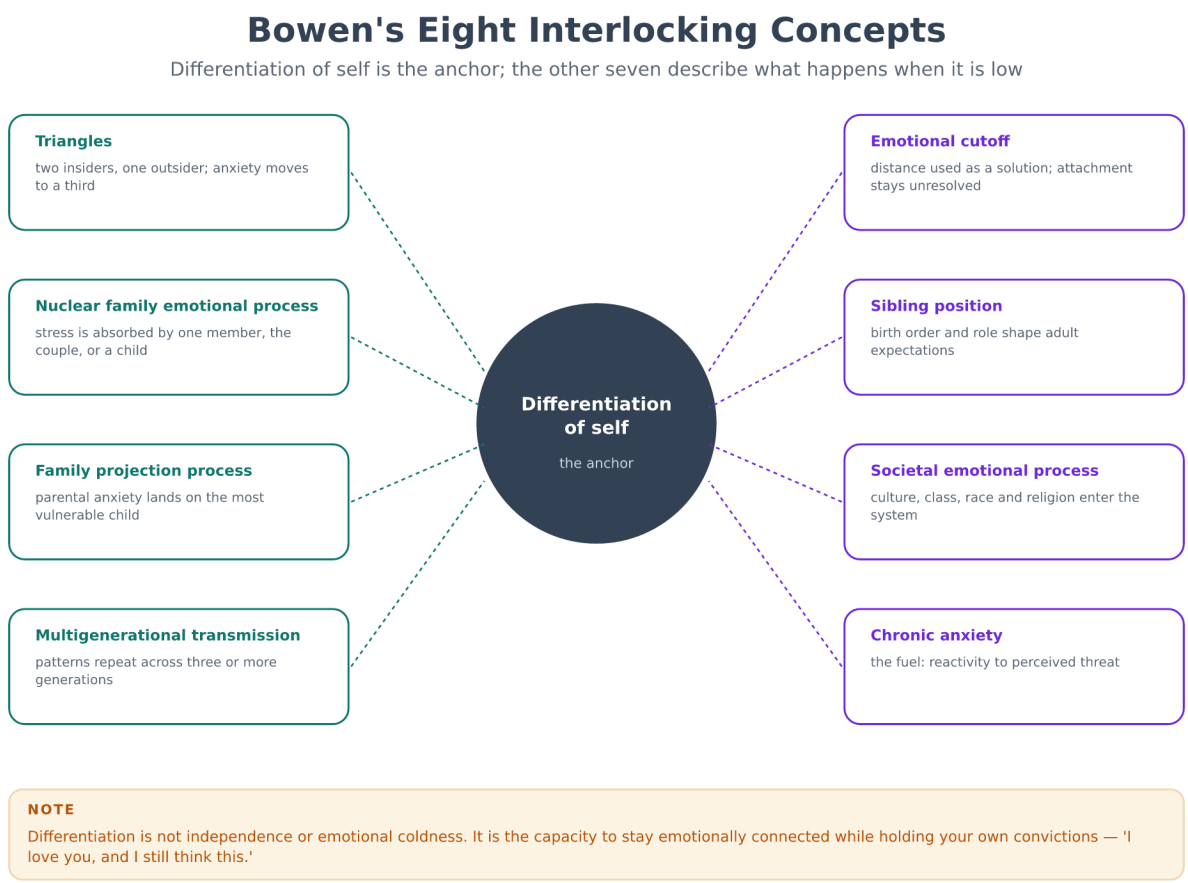


Figure 3.1 — Bowen's eight interlocking concepts

Core concepts. Differentiation of self (the capacity to stay connected while thinking for oneself); triangles; the nuclear family emotional process (marital conflict, dysfunction in a spouse, projection to a child, emotional distance); family projection process; emotional cutoff; multigenerational transmission; sibling position; societal emotional process; chronic anxiety.

Therapy. The therapist is a **coach**, not a fixer. The work is largely with the most motivated person (often "the over-functioner"), who learns to stay in contact with difficult relatives while managing their own reactivity. Genograms are the primary tool; the goal is differentiation, not symptom removal.

Signature technique — detriangling. "When your mother criticises your sister, what happens inside you? What would it look like to stay interested without taking a side and without leaving?"

Contribution. Genuinely systemic over time; explains why patterns repeat; insists the therapist do their own family-of-origin work.

Blind spots. Slow; assumes a level of choice and mobility not everyone has; can under-weight structure, gender and economics; evidence is largely clinical rather than trial-based.

> **PRACTICAL**
> Triads. One person describes a triangle in their family. The therapist practises three moves: name the triangle, ask about the client's own reactivity, and refuse to take a side. Rotate; the "client" rates how neutral (not cold) the therapist was.

Week 4 — Structural family therapy

The question. What is the organisation of this family, and how does its structure maintain the problem?

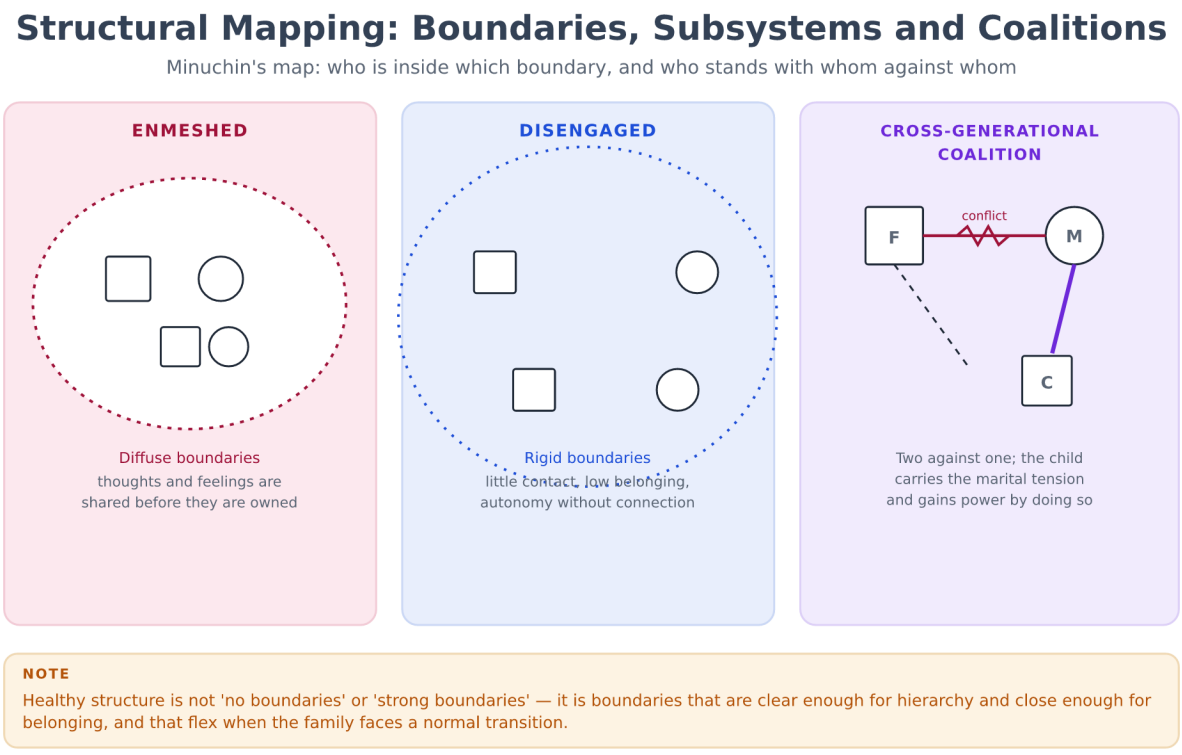


Figure 4.1 — Structural mapping: boundaries, subsystems, coalitions

Core concepts. Structure (the repetitive patterns of interaction that organise the family); subsystems; boundaries (clear, diffuse, rigid); hierarchy; alignment and coalition; enmeshment and disengagement; complementarity; the identified patient as the symptom-bearer of a structural problem.

Therapy. The therapist **joins** and then **challenges** the structure in the room. Change happens through doing, not talking: enactments, boundary-making, unbalancing, raising intensity, and marking hierarchy. Sessions are active and often directive.

Signature techniques.

- **Enactment:** "Say that to her, not to me. I'll listen."
- **Unbalancing:** deliberately supporting one member to shift the system's equilibrium.
- **Boundary-making:** "This is a conversation for the two of you; the children will wait outside."
- **Intensity:** "This is the fourth time you have laughed when your wife described the miscarriage. What is that laugh?"

Contribution. Fast, observable, highly teachable; strong evidence for structural family therapy in child and adolescent problems and in family-based treatment for substance misuse.

Blind spots. Risks imposing a middle-class norm of two-parent hierarchy; can be experienced as controlling; under-attends to inner life and to history.

> **CASE**
> The Adeyemi family. The structural reading: the parental subsystem is weak and the mother–daughter coalition crosses the generational line; the father is structurally peripheral and reacts with criticism when he is pulled in. First move: a session with the parents alone to plan one consistent rule and one shared evening, before any work with Amara. Second move: a joint session in which the parents tell Amara the plan together while the therapist blocks her attempts to negotiate with her mother alone.

Week 5 — Strategic and MRI brief therapy

The question. What is the family doing that keeps the problem going — especially their attempted solutions?

Core concepts. The problem is maintained by current interaction, not by the past; attempted solutions often persist because they "almost" work; the symptom may serve a function in the system; resistance is a response to the therapist's approach, not a property of the family; brief, focused, pragmatic.

Therapy. The therapist is an **active strategist** who designs interventions — tasks, directives, reframes, and sometimes paradoxical instructions. Sessions are few; between-session tasks do the work; termination is early by design.

Signature techniques.

- **Reframing:** "Your daughter's tantrums may be her way of keeping you two in the same room."

- **Ordeals:** a task that must be performed whenever the symptom occurs, making the symptom more costly than the change.
- **Paradoxical directive (with caution):** prescribing the symptom, under close supervision, in carefully selected cases.
- **Pretend technique:** "For one week, pretend to have the symptom on purpose for ten minutes a night."

Contribution. Broke the assumption that therapy must be long; emphasised the therapeutic power of therapist pragmatism and of clear, behavioural tasks.

Blind spots. Manipulation risks; weak evidence for paradoxical techniques; may be superficial in cases with trauma or severe mental illness; the "function of the symptom" can be speculative.

> **PRACTICAL**
> Design a one-week task for a family you know (disguised) that makes the current attempted solution impossible to continue, without instructing them to do the opposite in so many words. Present it; the group judges whether it is clear, doable, and safe.

Week 6 — Milan systemic therapy

The question. How do the family's beliefs and premises organise their behaviour — and how can curiosity disturb them?

Core concepts. The family as a meaning-making system; the **hypothesising, circularity and neutrality** stance; positive connotation (all behaviour is understood as serving the system); rituals; **circular questioning** (questions about differences and relationships rather than facts); the reflecting team and the "irreverent" later Milan style.

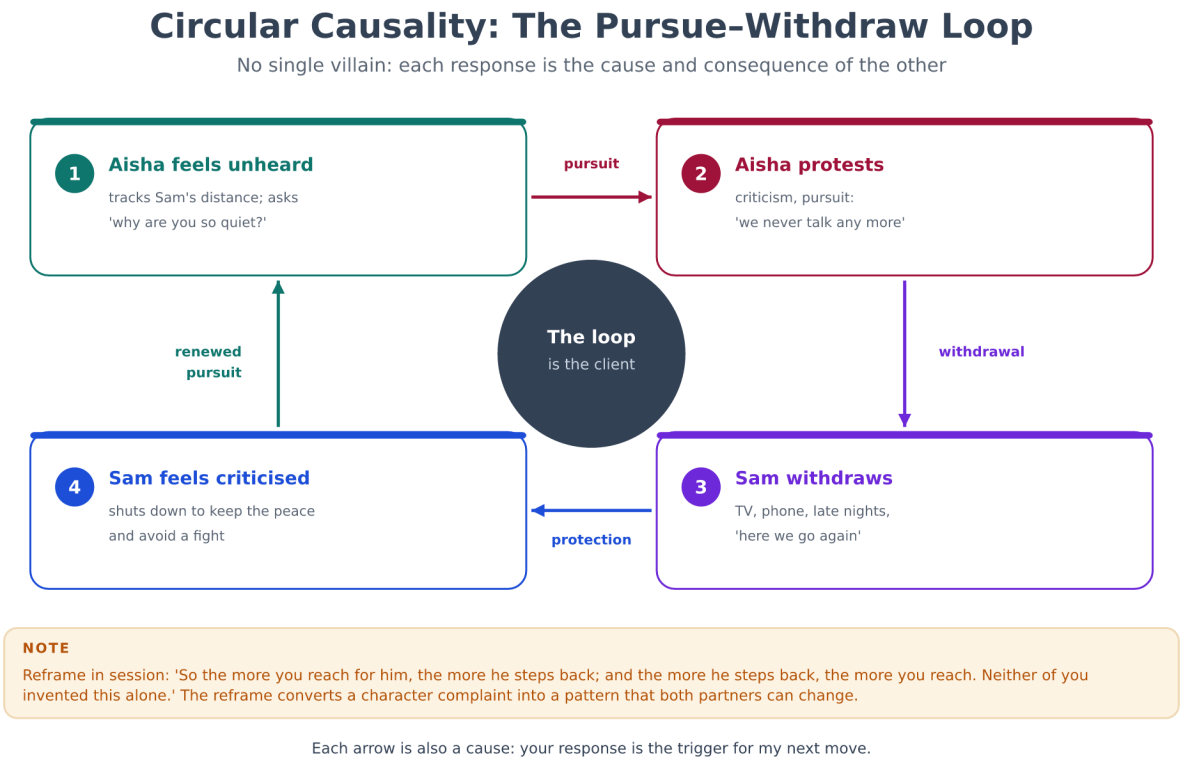


Figure 6.1 — Circular causality in practice

Therapy. The team observes behind a screen; the therapist asks circular questions; a consultation break produces an intervention that positively connotes the family's behaviour, often prescribing a ritual. The modern version is non-directive: the team's reflections are offered as multiple perspectives, not as verdicts.

Signature technique — circular questioning.

- "Who in the family is most worried about your mother's sadness?"
- "If your brother were here, what would he say your father does when you cry?"
- "On a scale from 1 to 10, how close are your parents? And how close would they say they are?"
- "What is the difference between the way your mother treats you and the way she treats your sister?"

Contribution. Gave the field its most transferable skill — the circular question — and its most useful epistemic stance, curiosity.

Blind spots. Can be experienced as evasive in crisis; neutrality is ethically dangerous in violence; team-based format is expensive.

PRACTICAL

Ten-minute interview with a volunteer "family" (two peers). You may ask only circular questions. An observer counts how many were genuinely circular (about difference/relationship) and how many were disguised facts. Aim for 80%.

Week 7 — Experiential and human validation models

The question. What would happen if this family could feel and say the truth to each other, right now?

Key figures. Virginia Satir (family sculpture, communication stances, self-worth, "the five freedoms"); Carl Whitaker (symbolic-experiential therapy, absurdity, the therapist's use of self); Kempler and the Gestalt-influenced tradition.

Core concepts. Growth rather than pathology; the present moment as the site of change; expression of authentic feeling; the therapist's own aliveness as a therapeutic instrument; family sculpture as non-verbal enactment of structure.

Signature techniques.

- **Family sculpture:** family physically arrange themselves to show the emotional map; then rearrange.
- **Communication stances** (Satir): placating, blaming, super-reasonable, irrelevant, and congruent.
- **Here-and-now enactment:** "Tell her now. I want to see what happens to your face when you say it."

Contribution. Humanised the field; insisted on warmth, humour and the person of the therapist. Many of its insights survive inside EFT and other emotion-focused models.

Blind spots. Dependent on the charisma and health of the therapist; thin manualisation; risk of emotional flooding without containment.

CASE

In a family session, the father has not cried in 20 years. Whitaker-influenced response: the therapist does not push the tears, but stays with the father's jaw and breathing, wondering aloud about what it costs to hold it. When the father finally speaks, the therapist keeps the room still long enough for others to respond. The intervention is pace, not technique.

Week 8 — Cognitive-behavioural and behavioural couple therapy

The question. What behaviours and beliefs maintain the distress, and how can they be changed directly?

Core concepts. Behavioural exchange and communication training (TBCT); cognitive restructuring of relationship standards and attributions; the role of selective attention (noticing negatives and missing positives); homework as the engine of change; skills deficit versus skills performance.

Therapy. Structured, time-limited, with agenda and homework. Skills are taught and rehearsed in the room, then practised between sessions. Integrative behavioural couples therapy addresses both behaviour and acceptance, with attention to culture and values.

Signature techniques.

- **Behavioural exchange:** each partner lists small, achievable caring behaviours; each does them daily.
- **Communication training:** speaker–listener with a talking object (see Figure 35, MFT 716).
- **Attribution retraining:** "When he is late, what do you assume? What else could it mean?"
- **Problem-solving:** define one problem, generate options, choose, trial, review.

Evidence. Among the strongest in the field; effective for relationship distress, with good evidence for depression treated in a couple format.

Blind spots. Can feel mechanistic; may under-weight emotion and attachment; assumes a degree of motivation and literacy; "skills" can be performed in session and abandoned at home.

> **PRACTICAL**
> With a peer, design a behavioural exchange contract: three behaviours each, defined observably, cheap, daily, and mutually agreeable. Then critique it: what could make this fail in a real family within two weeks?

Week 9 — Emotionally focused therapy

The question. What attachment need is underneath the anger, and how can the cycle be de-escalated so the bond can be repaired?

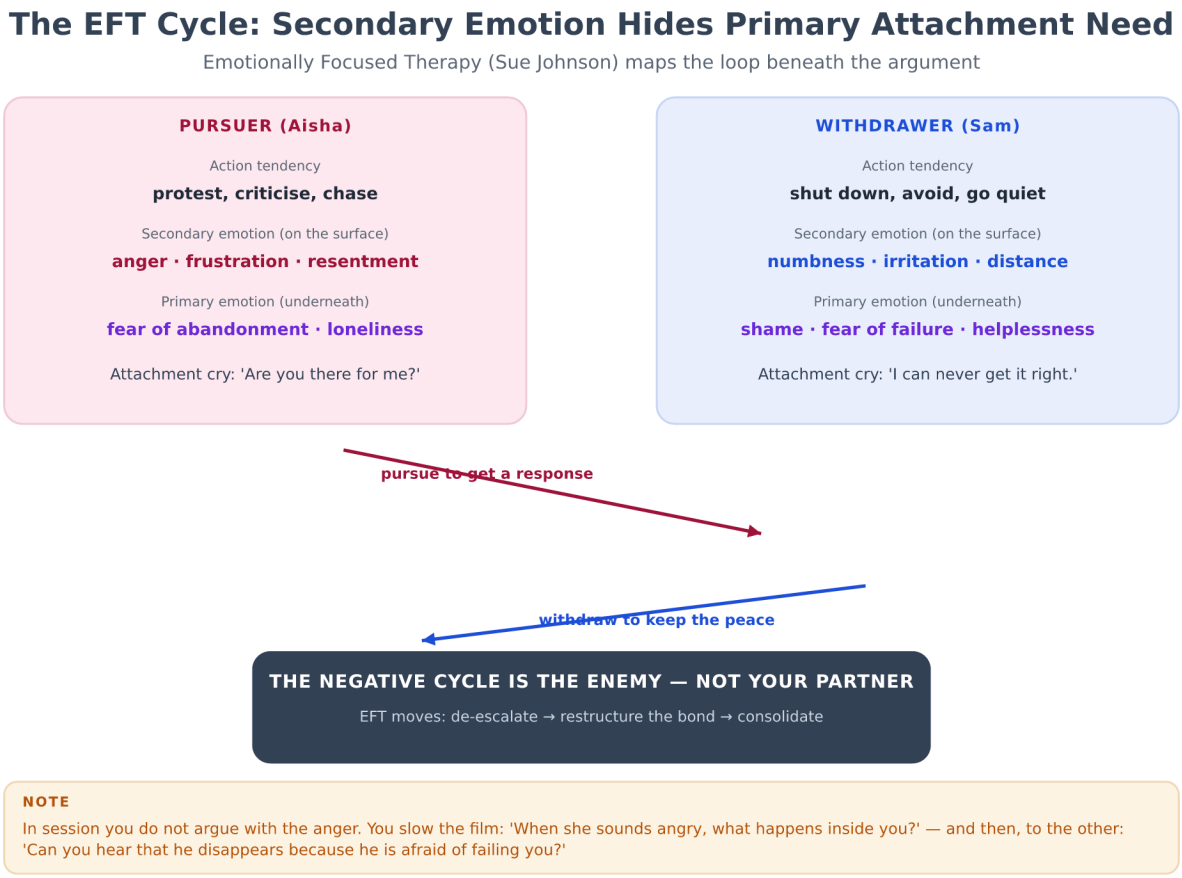


Figure 9.1 — The EFT cycle: secondary emotion and primary attachment need

Core concepts. Attachment theory as the map (see Figure 17); the negative cycle (pursue–withdraw, attack–defend, both-withdraw); primary versus secondary emotion; the "attachment cry"; softening events; the therapist as a process consultant and secure base.

Therapy in three stages. 1) **De-escalation** — identify the cycle, access primary emotion, reframe the problem as the cycle. 2) **Restructuring** — withdrawer re-engagement, pursuer softening, enactment of new responses. 3) **Consolidation** — integrate, problem-solve, prepare for recurrence.

Signature techniques.

- **Tracking the cycle:** "What happens inside you when she says that?"
- **Deepening primary emotion:** "Under the anger — is there something like fear of being alone?"
- **Enactment:** "Can you tell him that now, looking at him?"
- **Softening:** the pursuer's protest turns into an attachment expression, and the withdrawer responds.

Evidence. Among the most robust in couple therapy (over 30 trials, large effect sizes, durability in follow-up). Considered a first-line couple therapy for relational distress.

Blind spots. Requires a specific in-session emotional intensity and skills; less suited to ongoing coercive control; cultural calibration needed (the language of "needs" may feel self-indulgent to some families).

> **CASE**
> Aisha and Sam. Third session: the therapist slows a familiar argument to 20% speed. Aisha's anger is reframed as protest; Sam's silence as fear of failing. The first enactment — Aisha, quietly: "I get loud because I think you are leaving" — is the turning point. The next eight sessions are the work of making that statement safe to repeat at home.

Week 10 — Solution-focused brief therapy

The question. What is already working, and what would one small step look like?



Figure 10.1 — Solution-focused brief therapy in five moves

Core concepts. Solution talk versus problem talk; the miracle question; scaling; exceptions; coping questions; the client as expert; small, observable change; "if it works, do more of it".

Therapy. Conversational, non-normative, often 3–6 sessions. The therapist listens for competence and deliberately amplifies it; homework is usually "notice what works".

Signature techniques.

- **Miracle question:** "Suppose tonight a miracle happens and the problem is solved. What will you notice first tomorrow morning? Who else will notice?"
- **Scaling:** "On 0–10, where are you? What would one point higher look like this week?"
- **Exception questions:** "Tell me about a time the arguing stayed small. What were you doing differently?"
- **Coping question:** "With everything you have faced, how have you managed to keep going?"

Evidence. Good evidence for depression, with brief formats showing effects comparable to longer treatments in some trials; widely used in child protection and school settings.

Blind spots. Can bypass necessary grief or trauma work; risks ignoring risk; may feel invalidating when a family needs to tell the story of what happened.

> **PRACTICAL**
> Full miracle-question sequence in pairs (8 minutes), then a one-point scaling and a concrete task. Observer records: did the therapist ask for concrete behavioural detail, and did the client's posture change?

Week 11 — Narrative therapy

The question. Which story is dominating this family's life, and who wrote it?

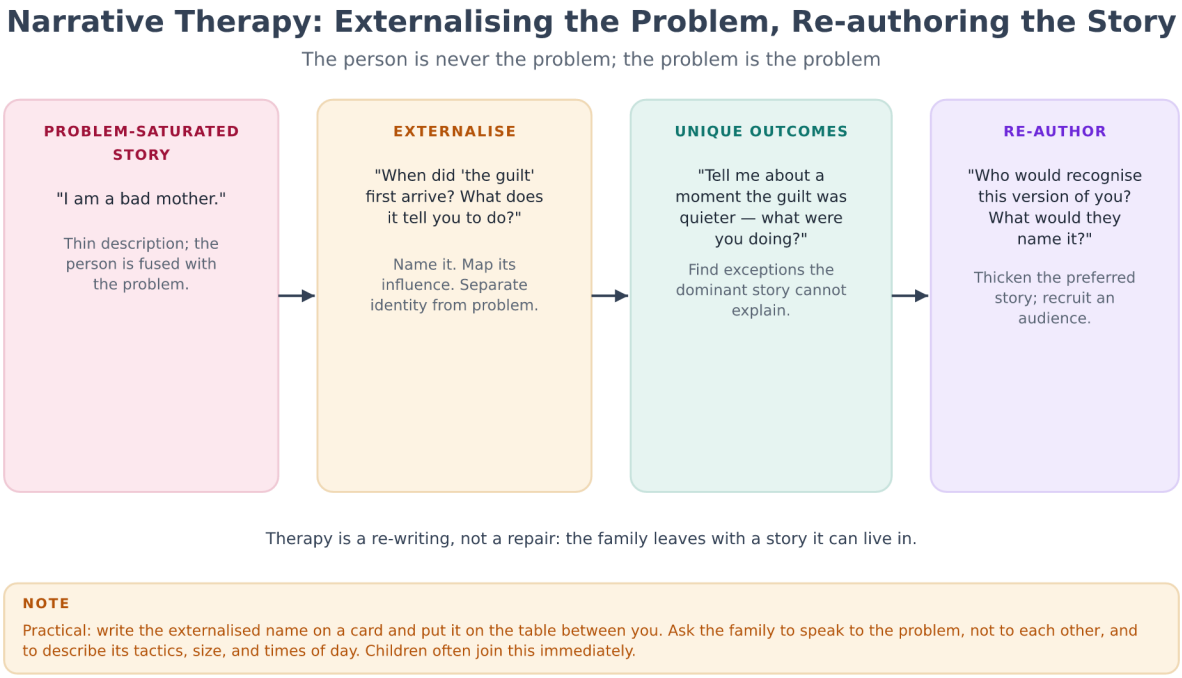


Figure 11.1 — Externalising and re-authoring the preferred story

Core concepts. The person is not the problem; the problem is the problem; dominant and preferred stories; thin versus thick descriptions; unique outcomes; externalising conversations; re-membering; definitional ceremonies; the therapist as decentred but influential.

Signature techniques.

- **Externalising:** "When did 'the guilt' first arrive in your life? What does it whisper to you?"
- **Mapping the influence:** "When does it have the most power? When does it lose power?"
- **Unique outcomes:** "Tell me about a moment when 'the shouting' was quieter."
- **Re-authoring:** "If you were to name this new version of yourself, what would you call it? Who would recognise it?"
- **Letter-writing** and **outsider-witness groups** to thicken the preferred story.

Contribution. The most culturally adaptable of the schools; central in work with children, with communities, and with people who have been defined by a diagnosis.

Blind spots. Can under-attend to material reality and risk; the linguistic sophistication can be inaccessible; evidence base is growing but smaller than EFT or CBT.

> **CASE**
> A mother referred by the school as "negligent". The therapist does not argue with the label but asks when "the exhaustion" took over, what it makes possible, what it prevents. Within thirty minutes the mother has described three occasions when she "fought the exhaustion and won", including a week when she arranged a rota with her sister. The referral, still true, is no longer the whole story.

Week 12 — Integration without incoherence

The question. How do you combine models so the client is not paying for your confusion?

Three legitimate forms of integration:

1. **Assimilative integration.** A home model, enriched by techniques from elsewhere. Most clinicians work this way. The home model keeps the formulation coherent.
2. **Sequential integration.** Different models at different phases — for example structural work to establish safety and hierarchy, then EFT for the couple's bond, then solution-focused work to consolidate.
3. **Theoretically integrated models.** Purpose-built blends (integrative behavioural couples therapy; attachment-based family therapy), coherent because they were designed together.

Rules for staying coherent.

Rule	Why
One formulation, one primary target	Prevents scattergun technique
Name the model out loud in supervision	Makes your reasoning auditable
Change model for a reason you can state	"Because the conflict escalates when we go near feelings"
Do not mix incompatible theories of change in one move	Reframing and behavioural contracting pull in different directions
Review at fixed intervals	Integration fails by drifting, not by decision

Trap to avoid: technique salad. Symptom-tracking from CBT, a miracle question, a family sculpture and an externalising conversation in one session does not make you integrative; it makes you unpredictable, and the family cannot tell what is expected of them.

> **ASSIGNMENT (30%)**
> One disguised case; two models; one formulation each; a reasoned choice of sequence; a predicted risk of the combination; a plan for how you would know it is not working.

Week 13 — Evidence, culture and critique

Model	Strongest evidence	Weakest evidence	Main cultural critique
Structural	Child/adolescent behaviour; substance misuse	Adult mood disorders	Imposes a particular family hierarchy
Bowen	Clinical tradition; modest outcome data	Few RCTs	Assumes mobility and choice; slow
Strategic/MRI	Some RCTs in child problems	Paradoxical techniques	Manipulative; may ignore trauma
Milan	Weak in classical form	Rituals, team format	Neutrality unethical in violence
Experiential	Moderate, largely process research	Manualisation	Charisma-dependent; middle-class
CB couple therapy	Strong	Severe relational trauma	Mechanistic; literacy-dependent
EFT	Strong, replicated	Coercive control	Individualist language of needs
Solution-focused	Moderate (depression, brief formats)	Severe mental illness	Can bypass grief and risk
Narrative	Growing; strong qualitative base	Large trials	Requires verbal fluency

The reflexive question. For each row, ask: whose family is this model describing? Models were developed largely with white, Western, middle-class, two-parent families in clinic settings. When you apply them to a polygynous household, a three-generation

compound, a family separated by migration, or a family whose authority structure is religious rather than developmental, you must translate — and say that you are translating.

Week 14 — Your theoretical position

Write 1,500 words: your home model, why it fits you, what it cannot do, the models you borrow from and under what conditions, and how you will know when to change your mind. Defend it in a ten-minute viva.

Integration: nine models on one page

Model	Problem is…	Change happens by…	Therapist is…	First move
Object relations	Unconscious re-enactment	Making the unconscious usable in the room	Container/interpreter	Notice the transference; interpret gently
Bowen	Undifferentiated anxiety across generations	Differentiation of self	Coach	Genogram + detriangling
Structural	Dysfunctional organisation	Restructuring in the session	Director/joiner	Enactment; strengthen hierarchy
Strategic	Attempted solutions	Interrupting the sequence	Strategist	Task, reframe, directive
Milan	Meaning premises	Curiosity disturbing belief	Curious team member	Circular questions
Experiential	Blocked feeling and authenticity	Present-moment expression	Real person	Sculpture; here-and-now
CB couple	Behaviour and attribution	Skills and rehearsal	Teacher/coach	Behavioural exchange
EFT	Attachment injury and cycle	Accessing primary emotion; enacting new responses	Process consultant	Track and reframe the cycle
Solution-focused	Problem-saturated attention	Amplifying existing solutions	Collaborator	Miracle/scale question
Narrative	Dominant story	Externalising and re-authoring	Decentred, influential	Externalising question

Glossary

Circular questioning — questions about differences and relationships that reveal pattern rather than fact. **Differentiation of self** — the capacity to stay emotionally connected while holding one's own position. **Enactment** — directing family members to speak to each other in the session rather than through the therapist. **Externalising** — locating a problem outside a person, as a thing with tactics and a history. **Negative connotation / positive connotation** — interpreting behaviour as serving the system, rather than as defective. **Paradoxical directive** — prescribing the symptom in order to change its function (used rarely, with supervision). **Reframe** — relabelling behaviour within a new frame that makes change possible. **Restructuring** — changing the family's organisation in the session through enactment and boundary work. **Scaling** — asking a family to rate a state on a numerical scale to make change visible and specific. **Unique outcome** — a moment when the dominant story did not hold, used as a foothold for a preferred story.

Self-assessment

Multiple choice

1. "The problem is the problem; the person is not the problem" is a principle of: (a) structural (b) narrative (c) Bowen (d) strategic

2. A therapist says, "For the next week, I want you to argue for exactly fifteen minutes at 8 p.m., then stop." This is closest to: (a) narrative (b) EFT (c) strategic (d) solution-focused

3. Positive connotation is associated with: (a) Milan (b) CB couple therapy (c) object relations (d) solution-focused

4. Which model places differentiation of self at its centre? (a) structural (b) Bowen (c) EFT (d) Milan

5. "Who would notice first if the problem were solved?" is a technique from: (a) structural (b) solution-focused (c) narrative (d) experiential

6. In EFT, the behaviour that conceals a primary attachment emotion is called: (a) enactment (b) secondary emotion (c) reframe (d) restructuring

7. Which model is most associated with family sculpture? (a) Satir (b) Haley (c) Minuchin (d) White

8. "Unbalancing" is a technique of: (a) structural therapy (b) solution-focused therapy (c) object relations (d) narrative therapy

9. Which model has the strongest replicated randomised evidence in couple therapy? (a) Bowen (b) Milan (c) EFT (d) experiential

10. Assimilative integration means: (a) blending theories equally (b) using a home model enriched by borrowed techniques (c) alternating models by session (d) refusing to commit to a model

Answer key

1 b · 2 c · 3 a · 4 b · 5 b · 6 b · 7 a · 8 a · 9 c · 10 b

Short answer

1. State the theory of change of structural therapy in two sentences, then name one case type where it is a poor fit and say why.
2. Write three circular questions you would ask a family in which a father has just lost his job.
3. What is the difference between positive connotation and reframing?
4. Give two reasons why narrative therapy suits work with children.
5. Name two incompatible assumptions that would make a structural-plus-narrative formulation incoherent.

Essay (choose one, 2,000 words)

1. Compare structural and EFT approaches to the same couple case. Which would you choose first and why?
2. "Neutrality is a luxury the powerful can afford." Discuss with reference to Milan therapy and domestic violence.
3. Critically evaluate the claim that theoretical integration is a sign of maturity rather than confusion.

Further reading

Comparative

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
- Gurman, Lebow & Snyder (Eds.). (2015). Clinical Handbook of Couple Therapy (5th ed.). Guilford.

Primary texts

- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
- Minuchin, S. (1974). Families and Family Therapy. Harvard University Press.
- Haley, J. (1987). Problem-Solving Therapy (2nd ed.). Jossey-Bass.
- Palazzoli, M. S., Cecchin, G., Boscolo, L., & Prata, G. (1980). Paradox and Counterparadox. Aronson.
- Satir, V. (1988). The New Peoplemaking. Science and Behavior Books.
- White, M. (2007). Maps of Narrative Practice. Norton.
- de Shazer, S. (1985). Keys to Solution in Brief Therapy. Norton.

- Johnson, S. M. (2019). Attachment Theory in Practice. Guilford.
- Epstein, N. B., & Baucom, D. H. (2002). Enhanced Cognitive-Behavioral Therapy for Couples. APA.

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